# Drug Coverage Policy

**Effective Date……06/15/2026**
**Coverage Policy Number……IP0702**
**Policy Title….Diabetes - Glucagon-Like Peptide-1 Agonists for Individual and Family Plans**

# Diabetes – Glucagon-Like Peptide-1 Agonists for Individual and Family Plans

- Bydureon BCise® (exenatide extended-release subcutaneous injection – AstraZeneca [obsolete 03/31/2025)
- Byetta® (exenatide subcutaneous injection – AstraZeneca, [brand obsolete 03/06/2025] generic)
- Mounjaro® (tirzepatide subcutaneous injection – Eli Lilly)
- Ozempic® (semaglutide tablets and subcutaneous injection – Novo Nordisk)
- Rybelsus® (semaglutide tablets – Novo Nordisk *[1.5 mg, 4 mg, and 9 mg tablets obsolete 03/20/2026]*)
- Trulicity® (dulaglutide subcutaneous injection – Eli Lilly)
- Victoza® (liraglutide subcutaneous injection – Novo Nordisk, generic)

***INSTRUCTIONS FOR USE***
*The following Coverage Policy applies to health benefit plans administered by Cigna Companies. Certain Cigna Companies and/or lines of business only provide utilization review services to clients and do not make coverage determinations. References to standard benefit plan language and coverage determinations do not apply to those clients. Coverage Policies are intended to provide guidance in interpreting certain standard benefit plans administered by Cigna Companies. Please note, the terms of a customer's particular benefit plan document [Group Service Agreement, Evidence of Coverage, Certificate of Coverage, Summary Plan Description (SPD) or similar plan document] may differ significantly from the standard benefit plans upon which these Coverage Policies are based. For example, a customer's benefit plan document may contain a specific exclusion related to a topic addressed in a Coverage Policy. In the event of a conflict, a customer's benefit plan document always supersedes the information in the Coverage Policies. In the absence of a controlling federal or state coverage mandate, benefits are ultimately determined by the terms of the applicable benefit plan document. Coverage determinations in each specific instance require consideration of 1) the terms of the applicable benefit plan document in effect on the date of service; 2) any applicable laws/regulations; 3) any relevant collateral source materials including Coverage Policies and; 4) the specific facts of the particular situation. Each coverage request should be reviewed on its own merits. Medical directors are expected to exercise clinical judgment where appropriate and have discretion in making individual coverage determinations. Where coverage for care or services does not depend on specific circumstances, reimbursement will only be provided if a requested service(s) is submitted in accordance with the relevant criteria outlined in the applicable Coverage Policy, including covered diagnosis and/or procedure code(s). Reimbursement is not allowed for services when billed for conditions or diagnoses that are not*

*covered under this Coverage Policy (see "Coding Information" below). When billing, providers must use the most appropriate codes as of the effective date of the submission. Claims submitted for services that are not accompanied by covered code(s) under the applicable Coverage Policy will be denied as not covered. Coverage Policies relate exclusively to the administration of health benefit plans. Coverage Policies are not recommendations for treatment and should never be used as treatment guidelines. In certain markets, delegated vendor guidelines may be used to support medical necessity and other coverage determinations.*

**Overview**
The glucagon-like peptide-1 (GLP-1) receptor agonists and the GLP-1/glucose-dependent insulinotropic polypeptide-1 (GIP) agonist addressed in this policy are indicated as adjuncts to diet and exercise to improve glycemic control in adults with **type 2 diabetes**.[1-7] Liraglutide, Mounjaro, Trulicity, and Bydureon BCise are additionally indicated for type 2 diabetes in patients ≥ 10 years of age.[1,2,6,7] Liraglutide, Ozempic, Rybelsus, and Trulicity also have labeled indications related to cardiovascular (CV) risk reduction in adults with type 2 diabetes.[4-7] Additionally, Ozempic is indicated to reduce the risk of sustained estimated glomerular filtration decline, end-stage kidney disease, and CV death in adults with type 2 diabetes and chronic kidney disease.[4] On January 30, 2026, the FDA approved a name change from Rybelsus 1.5 mg, 4 mg and 9 mg (previously "R2" Rybelsus) to Ozempic tablet 1.5 mg, 4 mg and 9 mg.[5,10] Rybelsus 3 mg, 7 mg, and 14 mg tablets remain available on the market at this time.

**Guidelines**
According to the American Diabetes Association Standards of Care in Diabetes (2026), pharmacologic therapy be guided by person-centric treatment factors including comorbid conditions, as well as treatment goals, and preferences.[8] Pharmacotherapy should be initiated at the time type 2 diabetes is diagnosed unless there are contraindications.

American Association of Clinical Endocrinology Consensus Statement: Algorithm for Management of Adults with Type 2 Diabetes (2026) recommends that pharmacologic therapy for adults with type 2 diabetes be selected using a comorbidities- and complications-centric approach, rather than a glucose-centric sequencing strategy alone.[9].

## Coverage Policy

**POLICY STATEMENT**
Prior Authorization is required for benefit coverage of the GLP-1 agonists and GLP-1/GIP agonist targeted in this policy. Of note there are other GLP-1 agonists not indicated for the treatment of diabetes that are not targeted in this policy. All approvals are provided for the duration noted below.

**Documentation:** Documentation is required where noted in the criteria as **[documentation required]**. Documentation may include, but is not limited to, chart notes, laboratory tests, claims records, prescription receipts and/or other information. All documentation must include patient-specific identifying information.

**I. Bydureon BCise, exenatide subcutaneous injection, liraglutide subcutaneous injection (generic to Victoza), Mounjaro, Ozempic injection, Ozempic tablets and Trulicity are considered medically necessary when the following is met:**

**FDA-Approved Indication**

1. **Type 2 Diabetes Mellitus.** Approve for 1 year if the patient meets the following (A):
   **A)** Diagnosis of Type 2 diabetes mellitus **[Documentation Required]**

**II. Rybelsus is considered medically necessary when the following is met:**

**FDA-Approved Indication**

1. **Type 2 Diabetes Mellitus.** Approve for 1 year if the patient meets **BOTH** of the following the following (A and B):
   **A)** Patient is 18 years of age or older
   **B)** Diagnosis of Type 2 diabetes mellitus **[Documentation Required]**

**III. Victoza is considered medically necessary when the following is met:**

**FDA-Approved Indication**

1. **Type 2 Diabetes Mellitus.** Approve for 1 year if the patient meets **BOTH** of the following (A and B):
   **A)** Diagnosis of Type 2 diabetes mellitus **[Documentation Required]**
   **B)** Preferred product criteria is met for the product(s) as listed in the below table(s)

**Individual and Family Plans:**

| Product | Criteria |
|---|---|
| **Victoza** (liraglutide subcutaneous injection) | The patient has tried the bioequivalent generic product, **liraglutide subcutaneous injection**, AND cannot take due to a formulation difference in the inactive ingredient(s) [for example, difference in dyes, fillers, preservatives] between the brand and the bioequivalent generic product which, per the prescriber, would result in a significant allergy or serious adverse reaction. |

**Conditions Not Covered**

**Glucagon-Like Peptide-1 Agonists for any other use is considered not medically necessary, including the following (this list may not be all inclusive; criteria will be updated as new published data are available):**

1. **Weight Loss Treatment**. Saxenda (liraglutide subcutaneous injection) contains the same chemical entity as liraglutide (Victoza, generic) and is indicated at a higher dose for chronic weight management. Wegovy (semaglutide tablets and subcutaneous injection) contains the same chemical entity as Ozempic and is indicated at a higher dose for chronic weight management. Zepbound (tirzepatide subcutaneous injection) contains the same chemical entity as Mounjaro and is indicated at the same doses for chronic weight management. The GLP-1 agonists and GLP-1/glucose-dependent insulinotropic polypeptide-1 agonist in this policy are not FDA-approved for weight loss in a patient who is overweight (body mass index [BMI] ≥ 27 kg/m$^2$) or obese (BMI ≥ 30 kg/m$^2$) without type 2 diabetes.
   Note: If the patient has type 2 diabetes, refer to FDA-Approved Indication.

2. **Type 1 Diabetes Mellitus.** None of the glucagon-like peptide-1 (GLP-1) agonists or GLP-1/ glucose-dependent insulinotropic polypeptide-1 agonist are indicated for patients with type 1 diabetes. [1-7] Addition of GLP-1 receptor agonists to insulin therapy resulted in small

(0.2%) reductions in hemoglobin $A_{1c}$ among patients with type 1 diabetes compared with insulin alone.[8]

3. **Prediabetes/Diabetes Prevention.** GLP-1 agonists and the GLP-1/ glucose-dependent insulinotropic polypeptide-1 agonist are not indicated in a patient with elevated blood glucose who does not have type 2 diabetes. The American Diabetes Association Standards of Care (2026) recommend consideration of metformin for the prevention of type 2 diabetes in adults at high-risk of type 2 diabetes, and in individuals with prior gestational diabetes mellitus. First-line recommendations to prevent or delay type 2 diabetes are lifestyle and behavioral modification (e.g., nutrition, physical activity, sleep).[7] Further, metformin has the longest history of safety data as a pharmacologic therapy for diabetes prevention.
   Note: If the patient has type 2 diabetes, refer to FDA-Approved Indication.

4. **Metabolic Syndrome.** The GLP-1 agonists and the GLP-1/glucose-dependent insulinotropic polypeptide-1 agonist are not indicated in a patient with metabolic syndrome who does not have type 2 diabetes.
   Note: If the patient has type 2 diabetes, refer to FDA-Approved Indication.

5. **Concomitant Use with Glucagon-Like Peptide-1 (GLP-1) Agonists or GLP-1/ Glucose-Dependent Insulinotropic Polypeptide (GIP) Agonist.** The GLP-1 agonists and the GLP-1/GIP agonist should not be combined with each other or with any other GLP-1 agonists or GLP-1/GIP agonist. There are other GLP-1 and GLP-1/GIP products not included in this policy that are FDA-approved for weight loss and are not indicated for type 2 diabetes.
   Note: Examples of other GLP-1 agonists not included in this policy include but are not limited to Foundayo (orforglipron tablets), Saxenda (liraglutide subcutaneous injection) and Wegovy (semaglutide tablet and subcutaneous injection). An example of a GLP-1/GIP agonist not included in this policy is Zepbound (tirzepatide subcutaneous injection).

## References


1. Mounjaro® subcutaneous injection [prescribing information]. Indianapolis, IN: Lilly; December 2025.
2. Bydureon BCise® subcutaneous injection [prescribing information]. Wilmington, DE: AstraZeneca; May 2025.
3. Byetta® subcutaneous injection [prescribing information]. Wilmington, DE: AstraZeneca; May 2025.
4. Ozempic® subcutaneous injection [prescribing information]. Plainsboro, NJ: Novo Nordisk; January 2025.
5. Ozempic® tablets and Rybelsus® tablets [prescribing information]. Plainsboro, NJ: Novo Nordisk; January 2026.
6. Trulicity® subcutaneous injection [prescribing information]. Indianapolis, IN: Lilly; May 2025.
7. Victoza® subcutaneous injection [prescribing information]. Plainsboro, NJ: Novo Nordisk; May 2025.
8. American Diabetes Association. Standards of care in diabetes – 2026. *Diabetes Care*. 2026;49(Suppl 1):S1-S371.
9. Samson SL, Vellanki P, Blonde L, et al. American Association of Clinical Endocrinology consensus statement: algorithm for management of adults with type 2 diabetes – 2026 update. *Endocr Pract.* 2026;32:473-518..
10. Data on file. Ozempic (semaglutide) Tablets Availability. Plainsboro, NJ: Novo Nordisk; February 9, 2026.

## Revision Details

| Summary of Changes | Review Date | Effective Date |
|---|---|---|
| New policy | 10/10/2024 | 12/01/2024 |
| **Removed** the metformin requirement from all products.<br>**Removed** preferred product requirements from Liraglutide, Mounjaro, Ozempic and Rybelsus.<br>**Updated** the Victoza preferred product requirement to a Multi-Source Brand approach. | 02/13/2025 | 04/01/2025 |
| **Added** generic exenatide to the policy to follow Byetta criteria | 05/01/2025 | 06/01/2025 |
| Conditions Not Covered<br>**Prediabetes/Diabetes Prevention.**<br>**Updated** the statement with information from the 2025 American Diabetes Association Standards of Care. | 06/12/2025 | 08/15/2025 |
| **Conditions Not Covered.**<br>Updated the Weight Loss Treatment statement. | 01/29/2026 | 04/01/2026 |
| **Updated** the policy and documentation statements.<br><br>Throughout the policy, Foundayo was added as an example of a glucagon-like peptide-1 (GLP-1) receptor agonist. | 04/23/2026 | 06/15/2026 |
| Ozempic tablets were added to the Policy.<br>**Policy Statement:** The Note listing the specific glucagon-like peptide-1 (GLP-1) agonists not targeted in this Policy was revised to state that there are other GLP-1 agonists not indicated for the treatment of diabetes that are not targeted in this Policy.<br>**Tye 2 Diabetes Mellitus:** The list of agents was updated to add Ozempic tablets; prior reference to Ozempic was updated to Ozempic injection. | 05/21/2026 | 06/15/2026 |

The policy effective date is in force until updated or retired.

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